



WORKPLACE SAFETY AND INSURANCE APPEALS TRIBUNAL

DECISION NO. 1373/11

BEFORE:

J.G. Bigras : Vice-Chair
M.P. Trudeau: Member representative of Employers
K. Hoskin : Member Representative of Workers

HEARING:

June 23, 2011 at Thunder Bay
Oral

DATE OF DECISION:

February 3, 2012

NEUTRAL CITATION:

2012 ONWSIAT 241

DECISION UNDER APPEAL:

WSIB Appeals Resolution Officer S. McKinnon decision
dated September 4, 2007

APPEARANCES:

For the worker:

Self-represented

For the employer:

WSIB account closed

Interpreter:

N/A

Workplace Safety and Insurance
Appeals Tribunal

505 University Avenue 7th Floor
Toronto ON M5G 2P2

Tribunal d'appel de la sécurité professionnelle
et de l'assurance contre les accidents du travail

505, avenue University, 7^e étage
Toronto ON M5G 2P2

REASONS

(i) Introduction to the appeal proceedings

- [1] The worker appeals a decision of the Appeals Resolution Officer (ARO), which concluded that he was not entitled to additional benefits for disabilities related to an injury at work on October 5, 1998. The ARO rendered a decision on the written record, without an oral hearing.

(ii) Issues

- [2] The issues under appeal are as follows:
1. Quantum of the 10% permanent disability award for the low back assessed on May 2, 1990;
 2. Quantum of the reassessed 20% pension and the arrears date of January 28, 2006;
 3. Entitlement to Chronic Pain Disability (CPD) award;
 4. Entitlement to a clothing allowance from 1997 to 2004;
 5. Entitlement to lost time benefits after May 7, 2005;
 6. Entitlement to lost time benefits while attending physiotherapy in 2007 and 2008;
 7. Entitlement to further Labour Market Re-entry (LMR) (formerly VR) Services.

(iii) Background

- [3] The following are the basic facts.
- [4] The now 51-year-old worker started as a labourer with a demolition firm on August 26, 1988. He was injured on October 5, 1988.
- [5] At the time of the injury, the worker was cutting the box on a truck body when the vehicle shifted and projected the worker to the ground. The worker was diagnosed with lumbar back muscle strain and left sciatica.
- [6] A permanent disability was diagnosed. On May 2, 1990, the worker was assessed a 10% pension. The pension was increased to 20% effective on January 28, 2006. The worker appeals the quantum and timing of these awards.
- [7] The worker could not return to his pre-injury employment and, given that the employer did not make suitable employment available, Vocational Rehabilitation Services (VRS) were provided. The Board agreed to sponsor the worker in a child and youth worker counselling program which he dropped in February 1995. Full benefits were discontinued and supplementary benefits were provided under section 147(4) of the 1989 Act, on the grounds that the worker could have returned to his pre-injury earnings had he cooperated with VRS.

[8] As a result of a Reinstatement Officer's decision, the Board re-opened VRS in September 1996. The worker completed the counsellor program and secured employment as an Addiction Counsellor at a halfway house. Following the loss of employment as a counsellor in 2004, the worker was treated for post-traumatic stress disorder (PTSD). Entitlement under the 1988 claim was denied for the PTSD. He also encountered problems of addictions and troubles with the law.

[9] The worker now requests the Board to institute a new LMR program. This has been denied by the ARO.

[10] The worker claimed a deterioration of his compensable condition, but entitlement to increased benefits was denied on the grounds that any deterioration of the back condition was due to a slipping incident at home on May 7, 2005. He appeals this issue claiming that he is entitled to total disability benefits after that date.

[11] Entitlement was also denied for CPD.

[12] The worker claims he has worn a Board-paid lumbosacral brace since 1989. He was granted a clothing allowance for different periods but was denied this benefit for the years from 1997 to 2003. He is claiming entitlement to a clothing allowance for that period.

[13] The worker also underwent physiotherapy after April 2007 and is requesting lost time benefits while undergoing this treatment.

(iv) Law and policy

[14] Since the worker was injured in 1988, the pre-1989 *Workers' Compensation Act* is applicable to this appeal. All statutory references in this decision are to the pre-1989 Act, as amended, unless otherwise stated. The hearing of the appeal commenced after January 1, 1998; therefore, certain provisions of the *Workplace Safety and Insurance Act, 1997* (the "WSIA") also apply to the appeal.

[15] Specifically, sections 45(1), 45(3), 50, 54 of the pre-1989 Act and section 147 of the 1989 Act govern the worker's entitlement in this case.

[16] Pursuant to section 126 of the WSIA, the Board stated that the following policy packages, Revision #8, would apply to the subject matter of this appeal:

- Policy packages: 10; 41; 45; 94; 95; 149; 300.

[17] We have considered these policies as necessary in deciding the issues in this appeal, in particular:

- *Operational Policy Manual* (OPM) Document No. 18-07-01, "Determining the Degree of Disability";
- OPM Document No. 18-07-02, "The Ontario Rating Schedule";
- OPM Document No. 15-03-01, "Recurrences";

- OPM Document No. 17-07-03, “Clothing Allowance”;
- OPM Document No. 15-04-03, “Chronic Pain Disability”;
- OPM Document No. 19-03-04, “Entitlement to LMR Plans.”

(v) The worker’s evidence

- [18] The worker stated in evidence that he was cutting the box from the main frame of a large truck when the box jackknifed and separated from the body. The worker was propelled forward and landed on all fours on the floor. He estimated the fall at about six feet and a half.
- [19] The worker’s back condition was assessed by Board physician Dr. J. Yeandle-Hignell on May 2, 1990, and a 10% pension was allowed with arrears to the date of the accident.
- [20] The worker stated that he not only suffered physical injuries but that he was traumatized by the fear that he was about to die. He stated that he had flashbacks and nightmares since that time. He states that doctors told him that he would have to live with the flashbacks. The frequency of these symptoms has decreased but he claims he had such a flashback three weeks before the hearing.
- [21] The worker gave evidence that the pain in his lower back and left leg has remained and in fact increased since the accident. He admits that he encountered alcohol and drug addiction problems and now takes only non-narcotic medication.
- [22] Following attendance at the Board’s Downsview Rehabilitation Centre (DRC) in 1990, the Board offered VRS assistance. The worker did not pursue the plan and program was dropped. However, in May 1992, a Reinstatement Officer directed that the VR file be re-activated. Following vocational assessment, the worker pursued his education through Confederation College (Thunder Bay) and Canadore College (North Bay) and obtained a diploma as youth and child worker.
- [23] In August 1998, the worker obtained employment as a counsellor at a drug and alcohol halfway house. During the worker’s employment at that institution, two of his clients committed suicide. The worker states that the two suicides set off post-traumatic stress causing lay-off from his job and eventual dismissal by the employer. The worker states that, in a legal process independent of the present case, his dismissal was found to be wrongful and he obtained some measure of redress.
- [24] An incident occurred on May 7, 2005, when the worker slipped on ice while descending stairs at home. The worker stated that he slipped and had to hold on to a handrail to prevent a fall. He subsequently complained of sharp pain shooting through his back and right buttock. The worker stated that his back condition was already on a deteriorating path and that this caused a further aggravation; therefore, the subsequent history of disabling pain resulted equally from the pre-existing condition and not from the May 7, 2005, slipping and near-fall incident. The Board found that the worker’s aggravated condition after May 7, 2005, resulted from the home incident of that date, and denied further lost time benefits.

[25] The worker appeals the denial of total temporary benefits after May 7, 2005.

[26] The worker also claims entitlement to temporary total disability benefits for the period during 2007 when he underwent physiotherapy.

[27] The worker also appeals the pension increase to 20% effective three months prior to an examination by Dr D.V. Hoffman on April 28, 2006.

[28] The worker also requests entitlement to a new LMR program. This was denied by the Board on the grounds the previous program which the worker completed had restored his pre-injury level of earnings.

(vi) Analysis

[29] Before we proceed with this analysis, the Panel wishes to make it clear that we were not persuaded by a number of the worker's representations which, rather than show the merits of his case, attack the actions and competence of a number of Board adjudicators, a manager, and a doctor.

[30] On this issue, the ARO explained her role in adjudicating issues. In her decision, the ARO stated:

...the administration of the claim file is not part of my reviews. The Appeals Branch reviews objections to the operating area decisions and determines whether these decisions are correct in accordance to the facts and circumstances of each case and applicable policy and legislation.

[31] The Appeals Tribunal takes the "de novo" approach to its considerations of workers' appeals, meaning that we are not actually "reversing" or "confirming" the ARO decision but we are making a new decision based on the balance of all the evidence as seen by the Tribunal decisions-makers.

[32] On the issue of considering the evidence, we note the words found in the Tribunal's Pension Assessment Leading Case, *Decision No. 915*, (May 22, 1987) 7WCAT1, at page 80:

Medical issues are at bottom factual issues and, like other factual issues in non-criminal proceedings, they fall to be decided on the balance of probabilities. The question is: on the basis of all medical evidence received, what seems most likely to be the answer?

[33] Therefore, we have considered this case from a perspective of all the evidence at hand without judging the actions of any individuals involved. Later in this decision, we will also look at the role of Board physicians in pension assessments.

(a) Was the 10% pension awarded May 2, 1990 correct?

[34] At relevant times in this case, permanent disability benefits were governed by section 45 of the *Workers' Compensation Act* S.O. 1984, c. 58, which stated as follows:

45(1) Where permanent disability results from the injury, the impairment of earning capacity of the worker shall be estimated from the nature and degree of the injury, and

the compensation shall be a weekly or other periodic payment during the lifetime of the worker or other such period as the Board may fix, of a sum proportionate to such impairment not exceeding in any case the like proportion of 90 percent of his average earnings during the twelve months immediately preceding the accident or such lesser period as he has been employed.

[35] In *Decision No. 915*, the Panel noted that section 45(3) provided that the Board may compile a rating schedule that may be used as a guide in determining the compensation payable in permanent disability cases. This is at the origin of the Ontario Rating Schedule (ORS) which is set out in OPM Document No. 18-06-02 listed earlier in this decision. As stated in OPM Document No. 18-07-01 “Determining the Degree of Disability,” the ORS adopted the Permanent Partial Disability Evaluation of the Association of Workers’ Compensation Boards of Canada in its report of September 1, 1984.

[36] The ORS allows a 30% pension for the complete immobility of the lumbar spine. Interpreted in terms of an award, the rating should relate to the impairment of earning capacity that the average person suffers when his or her back is completely immobile.

[37] In this case, the Board examiner estimated that the worker’s lumbar back had caused the loss of 10% of his capacity, therefore, one-third of his capacity. The Panel in this case is called to determine whether or not this is a correct evaluation of the worker’s loss of earning capacity due to his injury.

[38] *Decision No. 915* found that Board doctors were the persons more capable of giving a uniform interpretation of the ORS because they are relying on unwritten criteria and in-house judgment and experience. Using their own instead of Board criteria, non-Board doctors would be providing assessments made using different assessments. This would create a system of compensation which the Legislature could not have intended. This does not mean that independent physicians’ diagnoses and opinions which are considered as evidence going to the nature of the medical issues.

[39] For the above reasons based on uniformity of pension assessments, the Panel in *Decision No. 915* held that individual panels will have to ask what, relative to the Rating Schedule benchmarks, is the Panel’s estimate of the usual impairment of the earnings capacity of this injury?

[40] On May 2, 1990, Board Physician Dr. J. Yeandle -Hignell examined the worker and recommended a 10% pension with arrears to the time of the accident.

[41] Following is Dr. Yeandle-Hignell’s report:

History:

On October 5, 1988, this 29 year old labourer fell and landed on his hands and knees twisting his back. He was diagnosed initially as muscle strain with left sciatica and x-rays of the lumbar spine were reported as normal.

The worker was seen by Dr. Porter on March 14, 1989 and felt that there was a mechanical type of low back problem and arranged for bone and CT Scans which showed no evidence of disc prolapse or spinal stenosis but did indicate minimal degenerative

changes in the posterior facet joints of the lower two levels. The worker was referred to D.R.C. and physiotherapy was continued.

Following his workup at D.R.C., it was concluded that although there was a normal CT scan there did appear to be nerve root irritation on the left. It was noted that the worker was preoccupied with his pain and disability and seemed to amplify the former and to display a drug seeking behaviour. Surgery was not suggested but he was discharged to modified work with restrictions. The major diagnosis was low back strain and a rating was recommended. The worker was discharged from Downsview on December 6, 1989.

Since that time, the worker's condition appears unchanged and his last doctor's report was on April 10 of this year with a diagnosis of back injury and no improvement was anticipated.

At the start of the interview, the worker expressed dissatisfaction with the account of his accident and related it as follows: "I was standing on a frame, one foot on the box, spread eagle, I lifted up my right foot moving it back when the frame broke loose and flipped me up into the air. I came down like Superman. As I came down, the frame was coming up and knocked my feet over my head and I twisted violently away and landed like a cat."

Present Complaints:

Today, he is complaining of continuous aching in the low back and left buttock and also complains of burning and numbness in the left heel. He avoids bending and lifting and any twisting movements particularly to the left. He finds sitting difficult and can only manage 15 to 30 minutes at a time. He finds that walking two to three blocks gives him some relief. He is unable to play hockey, baseball, hunt or fish, which are activities which he indulged in frequently previous to the accident. His wife has left him and he feels that this is related to his back injury.

He is on no medications (says he has been in A.A. and relates it to drug and alcohol abuse brought on by the injury). He finds hot tubs are helpful. He has a back brace but does not wear it very much.

Physical Examination:

On examination, the worker is a well-muscled young man. The contours of the back are normal. On palpation, there is tenderness over the mid-lumbar spine and over the upper part of the left gluteus muscle. There is spasm in the left paralumbar musculature.

Flexion of the lumbar spine is at half normal. Extension is approximately a quarter of normal. Lateral flexion is three-quarters of normal on each side. Rotational movements are carried out slowly with discomfort at the limits of movements which are full. He can walk on his toes and heels and perform half a squat and recovery. Straight leg raising is to 70 degrees on the right and 40 degrees on the left limited by pain in the left buttock area. The bowstring sign is negative. Manipulation of the hip revealed some discomfort with rotational movements on the left. Power in the left lower limb is reduced because of pain and there is half an inch of wasting in the left quadriceps muscles. Knee and ankle jerks are brisk and the plantar responses are down going. There is some slight sensory loss to pin prick around the area of the left heel.

Diagnosis: Mechanical low back pain.

[42]

First, the Panel notes that the examining physician quotes the worker directly as to the nature of the accident giving details of the projection into the air and his fall landing on all fours. He also refers to Dr. Porter's diagnosis of evidence of sciatica, and mechanical type of injury.

He also refers to the medical evidence of bone and CT scans which showed no evidence of disc prolapse or spinal stenosis. The examining physician refers to the worker's examination at the DRC in August 1989 and the April 10, 1990 diagnosis.

[43] We have not found any evidence which would show that Dr. Yeandle-Hignell underestimated the worker's condition. Radiological evidence showed a mechanical back condition without any signs of disc or spine involvement. The worker argued that the examining physician did not consider leg problems which he suffered; however, we note that there was a worker's report dated January 12, 1989, which mentioned that he experienced pain in his heel, upper leg and lower back. These are mentioned in Dr. Porter's report of March 13, 1989, appearing in the record. Tests results from the DRC mention left buttock pain and left leg weakness, pain and numbness at times.

[44] On this evidence, showing mechanical low back and leg problems with no spinal or disc involvement, but pain extending to the left buttock and leg pain, we cannot doubt the correctness of Dr. Hingle-Hignell's 10% evaluation of the worker's disability in May 2, 1990. In making this finding, we are keeping in mind that the 10% figure represents a disability equal to one-third a totally immobile lower back.

[45] We are not questioning the Board's practice of awarding arrears to the date of the accident in this case.

(b) Was the 20% pension and arrears date of January 28, 2006, correct?

[46] On August 22, 2007, the ARO addressed a memo to Dr. A.D. Kanalec, a Board Appeals Medical Consultant. The ARO was seeking the physician's opinion on the quantum of the original pension, the deterioration of the work injury in view of the worker's home accident of May 7, 2005. The ARO also asked whether or not a pension reassessment was appropriate at that time.

[47] Later in this decision, we will look at the issue of the relationship between the accident of May 7, 2005, and the worker's subsequent condition. At this point we are considering the pension quantum and the arrears date of January 28, 2006.

[48] First, it must be said that the ARO's request memo of August 22, 2007, reviewed in detail the worker's medical history from the date of the original injury in 1988 to the date of the memo. She also reviewed the worker's arguments and his personal interpretation of the evidence. She carefully reviewed the diagnoses resulting from the accident, the diagnoses at the DRC in 1990, the acquisition of a back brace, and the views of family physician Dr. B. Bowerman, the radiological studies and the specialists' examinations, as well as the physiotherapy treatment received.

[49] Dr. Kanalec's report to the ARO, dated August 23, 2007, set out the history of the worker's condition as follows:

MEDICAL REVIEW:

I have read the entire file as well as your referral memo as well as relevant portions of the claim file.

According to the Employer's Report of Injury/Disease a truck frame broke and twisted overbalancing the worker. The worker was cutting a metal box from the frame of a Euclid truck that was on its side. The rusted metal let loose. There was injury to the low back, left leg and he had bruised kidneys.

Doctor's First Report from Dr. S. Araf noted that the worker felt back pain, pain in the left buttock and left leg, reflexes were normal, range of motion was decreased in flexion and extension. There was no bony tenderness and the diagnosis was muscle strain and left sciatica. He was prescribed analgesic medication. The Emergency Report from the Atikokan General Hospital noted that the worker fell at work yesterday, tumbled but there was no loss of consciousness, the back pain slowly increased with pain in the left buttocks, and a bruised left heel. Reflexes were normal. Straight leg raising was normal. There was decreased range of motion, flexion, extension and bending.

Doctor's Progress Report October 18, 1988 noted tenderness left sacroiliac joint, left sciatica and normal reflexes. According to the family physician Dr. Araf December 15, 1988, the worker remained tender over the left SI joint and was referred to Thunder Bay for further physiotherapy and manipulation.

January 13, 1988 Doctor's Progress Report noted the worker was improving. He still had some soreness and a tender left SIJ.

Report from the Intercity Orthopaedic and Sports Medicine Clinic March 14, 1989 from Thunder Bay indicated that [the worker] appears to have mechanical low back pain and further assessments were requested including a CT scan and blood work.

X-rays of the cervical, thoracic and lumbar spines dated March 14, 1989 from the Atikokan General Hospital were normal. Bone scan results from McKellar Hospital April 27, 1989 revealed a slight increased uptake in the lower back. This could be a facet but there was not a corresponding abnormality seen in either the sagittal or coronal views. Therefore it is not considered to be confirmatory of facet joint arthritis.

CT scan lumbar spine April 28, 1989 noted no evidence of disc prolapse. The scan was from L3 to S1. All lumbar levels were normal. The opacity of the lumbar spinal canal was within normal limits with no evidence of spinal stenosis.

The worker was admitted to the Downsview Rehabilitation Centre October 17, 1989. He presented as a muscular fellow, well nourished but not noticeably overweight. The diagnosis was lower back strain and the secondary diagnosis was left S1 radiculopathy. The admission diagnosis was left S1 radiculopathy. He had an orthopaedic consultation by Dr. G. Dale October 31, 1989. Dr. Dale said on the balance "I do not think that he does not have evidence of disc herniation. His problem is contusion to his lower back from which he has been very slow to recover but the treatment he is getting now is the treatment he requires and he should be able to get back to work again".

Physiotherapy Discharge Report from DRC noted that treatment unfortunately was unsuccessful. In fact, subjectively he was worse complaining of more pain. Objectively his posture and gait improved and he demonstrated some improvement movement of his lower back which was very guarded on assessment.

Progress Report from Dr. Garland from Downsview Rehab Centre October 31, 1989 stated that "[the worker] was wrapped up into his pain and that everything he tried so far had made him feel worse. The McKenzie routine, TENS, short wave diathermy and immobilizations. He was also unhappy in the PT pool. Objectively, however, he was moving his back better and walking better. He made a fair effort on a very light program". Idarac was not helping and Tylenol #3 was not relieving his pain and he wanted a prescription for Percodan but Dr. Garland declined. He then went on to say that if he could get medication like Percodan other than through a doctor's prescription and at that point Dr. Garland decided not to prescribe anything for him at all.

The worker had EMG and NCT studies done by Dr. S. Garg November 17, 1989 and the electrodiagnostic studies failed to suggest an L5-S1 radiculopathy on the basis of nerve conduction studies. A follow-up report November 21, 1989 indicated that he had pain from the electrodiagnostic studies and he stated that “he had never experienced pain like this in his life before” and thought that the electrodiagnostic studies were responsible.

The worker was discharged from DRC on November 30, 1989. Conclusion – he gives a good description of left sided sciatica and on examination there seems to be nerve root irritation on the left. However, the CT scan is normal. There is no good objective evidence of a neurological deficit. He is preoccupied with pain and disability which seem to be amplifying the former and he displays drug seeking behavior and therefore feel that he would not do well with surgery. In the doctor’s opinion [the worker] was fit for modified work with restrictions suggested by the gymnast:

1. No bending
2. No lifting greater than 5 kgs and allowed to alternate position from standing to sitting.

Diagnosis low back strain, left S1 radiculopathy.

He had a permanent impairment examination dated May 2, 1990. This resulted in a 10% award with possible deterioration over time and the diagnosis being mechanical low back pain.

A report from Dr. Mukovy February 2, 1996 noted that the patient still complains of lower back pain and uses a back brace for sitting in his classes at school and for doing any physical activities. He is currently attending school in North Bay. Diagnosis – chronic muscular back pain. Reflexes, power, sensation and pulses were normal. Straight leg raising revealed 90 degrees in his right leg and 70 degrees in his left. There was no evidence of hip or sacroiliac disease.

A report from Dr. Bowerman, MD November 2, 2004 noted his back pain remains severe enough that he cannot stand for prolonged periods or sit for prolonged periods. In summary this gentleman has longstanding back pain and has been avoiding any form of manual work and was on a 10% pension. He is now facing criminal charges in the performing of some non-manual work that he had taken on and as such is no longer employed. As such he has developed severe depression. At the present time he is in hospital and having ECT. While he has become inactive the pain from his back has become even more severe.

Discharge Report from Lake of the Woods District Hospital in Kenora December 12, 2005 noted moderate chronic episode of major depressive disorder after having 3 ECT treatments. [The worker] decided he did not wish to have more. This man was described as having a fairly severe history of alcohol and polysubstance abuse which has been inconsistent and in full remission since the age of 31 until now.

I reviewed a report from Dr. Hoffman, orthopaedic surgeon April 28, 2006. Dr. Hoffman noted under the auspices of the WSIB he was actually trained as a drug and alcohol counselor and he was able to get himself off those substances. He worked from 1998 to 2003 in that profession and had to stop work. On examination there was significant guarding of the lower back with little flexion and extension available. Nonetheless reflexes are bilaterally equal and physiological. Motor power was satisfactory. There was no significant tenderness around the heel of the plantar fascial area. MRI of the lower back demonstrated collapse at both L4-5 and L5-S1 discs. At L4-5 a right sided disc prolapsed and at L5-S1 with approach of the right L5 nerve root. He has a congenitally small spinal canal but no spinal stenosis. Facet arthrosis is developing. It is obvious that he is developing a significant chronic pain syndrome.

Physician’s Progress Report April 23, 2007 noted chronic back pain from discs.

The Claims Adjudicator requested a medical opinion on the claim deterioration. The opinion was that the ongoing problems appear to be the result of DDD of the lumbar

spine and the manager was not in agreement with this as he noted that the back symptoms increased after the slip. Entitlement to a Pain Clinic was allowed but apparently not acted upon by the treating doctor. A further medical opinion was obtained and it was in this part that the current findings of the L4-5 and L5-S1 could not be attributed to the remote compensable injury on the basis of this claim.

[50] Dr. Kanalec's medical opinion, expressed as answers to the ARO's questions. To the question from the ARO on whether or not a pension reassessment was warranted, Dr. Kanalec replied as follows:

The worker presents with both physiological and non-physiological findings. I can rate him based on the clinical findings on file and therefore an in person pension assessment would not be required. At this particular time I would increase the 10% compensable award to 20% with arrears 3 months prior to the report from Dr. Hoffman dated April 28, 2006. The findings of MRI are likely progressive degenerative changes and one could argue that these were triggered by the initial compensable accident and hence are compensable.

[51] The Panel's review of the medical file shows that, subsequent to the PD assessment of May 2, 1990, and a vocational assessment of June 21, 1992, the worker did not enter any medical records in his file until April 10, 2005, when Dr. Bowerman reported on the worker's complaints of continuing lower back pain and that he had been wearing a back brace for 8 to 10 years.

[52] On April 28, 2006, the worker was examined by orthopaedics specialist Dr. D.V. Hoffman. The report of that examination mentioned that the worker had been injured in 1988 and eventually was retrained, returned to work as a counsellor, developed drug and alcohol problems, left his job due to a stress disorder and his addictions. Dr. Hoffman mentioned the slipping incident in May 2005, and the subsequent chronic pain in the left low back region and more so in the right sacro-iliac area.

[53] Dr. Hoffman's report of ongoing symptoms and complaints continued as follows:

He has difficulty sleeping during the night and is frequently awakened. In the morning his back is stiff for about ½ an hour. During the daytime he says sitting is his most uncomfortable posture. Walking is his best although there is a limit to how far he can manage. He says with coughing and sneezing he has an increase in his back pain, but not the leg symptoms.

Treatment over the last year has consisted of Flexeril and Celebrex. He was tried on Morphine, but was afraid to take it because of his past history of addiction. He did start using a cane after the slip and fall in May. For many, many years, well before the May incident, he has been wearing an orthopedic type of back support. He tells me that he is also getting a bit of pain above the level of the support.

Investigations have included an MRI.

On functional inquiry he is on Remeron, Femotidine at nighttime. He is a nonsmoker with no cardiorespiratory symptoms. He is overweight at 245 pounds on a 5ft 10 inch frame. Bowel and bladder function are normal.

On examination there is significant guarding of the low back with very little flexion or extension available. Nonetheless, reflexes are bilaterally equal and physiological, motor power is satisfactory. There was no significant tenderness around the heel, the plantar fascia, etc.

He has been having some problems with his right shoulder as well and I only had an opportunity for a brief assessment of the shoulder. Examination showed a positive Kennedy-Hawkin's test, negative Neer test, but positive active compression, positive Scarf test.

The MRI of his shoulder showed mild AC joint osteoarthritis. There was no full thickness tear of the rotator cuff or muscle atrophy.

The MRI of his lower back demonstrates collapse of both L4-5 and L5-S1 discs with at L4-5 a right sided disc prolapsed. At L5-S1 again there is a right sided disc prolapsed with approach to the right L5 nerve root. He has a congenitally small spinal canal, but no additional spinal stenosis. Facet arthrosis is developing.

With respect to his back problem, the patient obviously has developed a significant chronic pain syndrome. I have had access to the report from Dr. Hignal, the WSIB Regional Medical Advisor, dated May 2, 1990, and there is considerable deterioration of this man's physical examination from that time to the present. In my opinion, this man needs treatment for his chronic pain syndrome. Because of his understandable reluctance to use opioids, it will be difficult I think to provide him with good pain control. He has never attending a Chronic Pain Program and that should certainly be recommended. It should also be funded through the WSIB since his current problems relate quite directly to that original injury.

With respect to the right shoulder, I would recommend one or two more steroid injections. He should also have a good session of physical therapy for the shoulder with anti-inflammatory modalities, remedial exercises, etc. I have told the patient that this is a very complicated problem to deal with in just one ½ hour. If you can institute some of these recommendations I will be happy to continue following him on my visits to Kenora.

[54] Dr. Hoffman's report served as the guide for Dr. Kanalec's recommendation to increase the worker's pension from 10% to 20%.

[55] Comparing the medical facts between the 1990 PD assessment and the 2006 diagnosis by Dr. Hoffman, we note that the worker suffered a mechanical low back pain in 1988. CT Scan at that time did not show any evidence of disc prolapse and only showed minimal degenerative damage, although nerve root irritation was suspected. Rotation was limited at 75% and leg raising was limited to 70 degrees.

[56] When examined by Dr. Hoffman in April 2006, the worker showed significant guarding of the low back with very little flexion or extension. MRI had shown prolapse of L4-4 and L5-SI discs. Facet arthrosis was developing.

[57] The Panel has no difficulty finding that the nature and degree of the worker's lower back injury had increased significantly from 1990 to 2006. Again, keeping in mind that the rating schedule attributes 30% for a completely immobile back, we are satisfied that the 20% award, representing a value of two-thirds of a complete immobility, fairly represents the worker's level of disability at the time of reassessment in May 2007.

[58] The worker also questions the correctness of the Board's decision to award the increase to 20% to January 28, 2006. This is three months prior to Dr. Hoffman's examination report on which the pension reassessment was based. Awarding three-months of arrears is a Board practice which recognizes that the change in condition did not occur abruptly on the date it was diagnosed. Unless additional medical evidence is available on that matter, the Board sets the date

of the change at three months before the examination on which the new pension quantum is based.

[59] In this case, the worker states that the low back condition was increasingly painful since 1990 and that he was prescribed medication which he took throughout the period at issue. He also states that he was wearing a back brace and that was another sign of higher disability than recognized by the 10% award.

[60] However, it must be said that the worker was able to work as a counsellor for more than five years during the period at issue. He lost his job in 2004 for reasons not linked to his back condition. Also, there is no record of any medical treatment during that period and reports by Dr. Hoffman and Dr. Bowerman confirm that the increased symptoms arose after May 2005.

[61] For those reasons, we are denying the worker's appeal of the quantum and effective date of his 20% pension that came into effect on January 28, 2006.

[62] Later in this decision, we will look at the worker's entitlement for the period from May 7, 2005.

[63] However, while we could not alter the Board's decision pertaining to the quantum of the worker's pension award, we note that Drs. Hoffman and Yeandle-Hignell state that the worker's disability is progressive. The worker also reports increasing pain and disability. The Panel therefore finds that, given that the last assessment was close to seven years ago, a new assessment is now in order and directs the Board to conduct such an examination. The pension determination resulting from this direction may be appealed by the worker.

(c) Is the worker entitled to benefits for CPD?

[64] Chronic Pain Disability is the term used to describe the condition of a person whose chronic pain has resulted in marked life disruption. Chronic Pain is pain with characteristics compatible with a work-related injury, except that it persists six or more months beyond the usual healing time for the injury.

[65] The above definitions are from Board OPM Document No. 15-04-03 "Chronic Pain Disability." This document also sets out the criteria which must all be met for entitlement:

1. a work-related injury occurred;
2. chronic pain is caused by the injury;
3. the pain persists six or more months beyond the usual healing time of the injury;
4. the degree of pain is inconsistent with organic findings
5. the chronic pain impairs earning capacity.

[66] There is no doubt that the worker sustained an accident at work on October 5, 1988, and that he has been suffering pain related to the injury since that time. The original diagnosis was of mechanical low back pain with some deterioration possible. A pension of 10% compensated for this organic condition. The worker could not return to his pre-injury work with the accident

employer and was eventually retrained and worked during about five years as a counsellor. We accept that the worker continued to experience some pain, he did not require medical attention for his low back until he suffered a recurrence on May 7, 2005. Shortly afterwards the worker was diagnosed with L4-L5 and L5-SI disc involvement. The worker's pension was increased to 20% in recognition of this organic condition. Therefore, it cannot be said that the degree of pain is inconsistent with the organic findings.

[67] In order to have entitlement it must be shown that all five criteria have been met. Absent evidence that the worker's pain was inconsistent with the organic findings, we cannot grant entitlement for CPD.

[68] We are aware of the diagnosis by Dr. Hoffman in his report of April 28, 2006, stating that the worker needed treatment for a chronic pain syndrome. However, as stated in *Decision No. 915*, the term "chronic pain" can include "long-lasting pain," a pain which goes on for an unexpected length of time but can be explained by physical symptoms, and "enigmatic" chronic pain which includes unexplained but real symptoms. It is this latter type of chronic pain which is included in the policy for Chronic Pain Disability which requires that "the degree of pain be inconsistent with organic findings." In his report of April 28, 2006, Dr. Hoffman diagnoses the source of the pain as disc prolapses, some arthritis and nerve root involvement. It is significant to note that, in a subsequent report dated August 7, 2006, Dr. Hoffman refers to "a history of chronic back injury..." and, later, to "chronic and persistent back pain..." We find that Dr. Hoffman's mention of a chronic pain syndrome refers to the long-lasting low back pain and not to a chronic pain disability which requires that the pain be inconsistent with the organic findings.

(d) Does the worker have entitlement to a clothing allowance?

[69] The Board allows clothing allowances for workers with permanent impairment to replace or repair clothing worn or damaged by wearing an assistive device or prosthesis.

[70] In this case, the worker was first prescribed a back brace on May 25, 1989. There is no further evidence on its use or damages to clothing or the device itself.

[71] The next mention of a back brace appears in the family physician's report dated July 31, 1995, which stated that the worker had been given a back brace by a specialist a year earlier and that he should continue to use the device but not on a steady basis to prevent atrophy.

[72] On February 7, 1991, allowance was granted up to May 1991. The worker was advised that further consideration would be given once it was shown that a brace was purchased and had been worn for a year. Subsequently, such allowances were made for the year from May 1991 and May 1992. The Board at that time had also allowed for a new brace. However, no further claim was received from the worker for clothing allowance.

[73] The Board paid for a new back brace on May 21, 1996. An application for clothing allowance was received in April 1998. The prescription was for the worker to use such a device only for prolonged sitting or performing house or yard work.

[74] No allowances for clothing replacement or damage were made from 1997 to 2004. In a letter dated April 10, 2005, family physician Dr. Bowerman stated that the worker required a back brace for eight to 10 hours a day. Clothing allowances were paid starting in 2004.

[75] The worker stated in evidence that he continued to wear a back brace for more than eight hours a day from 1997 to 2004. He did not apply for a clothing allowance because he believed that, having received this benefit in the past and given that he continued to wear a brace, he believed that the payment would be made automatically.

[76] Board OPM Document No. 17-07-03 "Clothing Allowance" states in its guidelines that "workers must apply annually for a clothing allowance by submitting a written application and information indicating the type of damage to the clothing."

[77] In a letter to the worker dated February 6, 1996, a Board Health Care Benefits Adjudicator outlined in length the Board policy. The adjudicator at that time allowed payment for repairs to the worker's brace and stated that consideration would be given to a clothing allowance following the wearing of the brace for one year.

[78] Upon application by the worker on April 21, 1998, the Board authorized a clothing allowance up to May 31, 1997. The worker was advised to re-apply in June 1998 for the allowance up to May 31, 1998. No further word was received from the worker until he mentioned the back brace issue in 2004, and re-applied on September 9, 2005.

[79] The Panel notes that the Board policy is clear in the matter of a worker's requirement to apply for a clothing allowance after each year of use. This is a benefit which depends on changing circumstances and it is appropriate to inform the Board on a timely basis. In this case the worker had been made well aware of the policy and the worker appears to have ignored it from 1997 to 2004. The worker's failure to contact the Board deprived the Board of the information necessary to make such an allowance such as the hours of use of the back brace, the type of damage encountered to the clothing and the cost of replacing these items.

[80] For those reasons, we are denying the worker clothing allowances for the years 1997 to 2004.

(e) Was the worker entitled to full benefits after May 7, 2005?

[81] Looking back at the history of benefits in this case, we note that the worker had received full assistance for a Vocational Rehabilitation (or Labour Market Re-entry) program and had been employed at a halfway house for about five years to February 2005. It is not disputed that he had reinstated his pre-injury earnings level.

[82] The worker lost his job at the half-way house after leaving employment due to Post Traumatic Stress Disorder. The worker states that PTSD resulted jointly from the compensable injury and the stress associated with two suicides of his clients at the halfway house. The issue of cause of the PTSD is not before us and the worker would have to return to the Board if he wants a determination on that issue. The issue of wrongful dismissal for which he received a civil settlement would likely be seen as a consideration.

[83] However, in the present case, the worker claims that he suffered an aggravation of his compensable injury on May 7, 2005, and that he was totally disabled afterwards and requests temporary total disability benefits.

[84] The worker gave evidence that, on May 7, 2005, he was coming down some stairs when he slipped. He had to retain himself by holding onto a railing while he dropped down some steps. He stated that he did not fall but twisted his body to keep from falling. The worker stated that he already experienced consistent pain in his back but that he felt a sharp shooting pain in his back, right buttock and left leg as a result of the near-fall of May 7, 2005. He stated that the pain would not go away.

[85] In a continuity report to the Board, dated April 8, 2006, the worker stated that he “slipped on some ice, twisted to keep my balance, and re-injured my back/left leg.” The worker added that he “had a chronic pain in (his) back/leg since (his) injury and the pain has greatly increased (May 7, 05)” The worker also stated that he had stopped work “being a landlord” but added that “after slipping and hurting my back/leg again, I was not able to work since May 7, 05.”

[86] The worker argued that he was totally disabled after the May 7, 2005, incident and that this was due to his injury of October 8, 1988. He claims that he has been unable to work and that he should be paid total disability benefits.

[87] In this case, it is clear to us that the aggravation was directly related to the incident at home on May 7, 2005. The worker was descending stairs when he slipped and twisted his back trying to hold on to a rail and preventing a fall. However, we cannot ignore the involvement of the compensable disability. Board physician Dr. Kanalec stated in her pension assessment of August 23, 2007, that “one could argue that (the MRI findings) are likely progressive degenerative changes....triggered by the initial compensable accident...”

[88] This is confirmed by Dr. Hoffman who stated as follows in his letter to the Board dated November 5, 2006:

...it is certainly my expert opinion that the likelihood of (the worker's) current problems in the low back and with chronic pain problems are directly related to the work-related injury which has been well documented

[89] Family physician Dr. Bowerman stated as follows in his report of October 12, 2006:

I am at a loss to understand how somebody who has a bad back from a compensable injury and reinjures their (sic) back with minor stress can be considered to have a condition unrelated to that injury.

[90] Board OPM Document No. 15-03-01 “Recurrences,” found in the record, states that permanently injured workers who suffer a recurrence may be entitled to temporary benefits if the recurrence causes temporary benefits for an increased disability. However, in this case, we do not find that the incident of May 7, 2005, caused such total disability. First, there is no record of any immediate treatment. There is no mention in the medical record of this incident until Dr. Hoffman's report of April 26, 2006, close to one year after the event. Also, the worker's report of such incident was not filled out before April 8, 2006. Moreover, in that latter report, the worker does not refer to any medical treatment for this alleged injury. In order to allow

temporary benefits it would have to be shown that the level of disability during the post-May 7, 2005, period was higher than the level recognized by the permanent disability level (pension). As confirmed earlier in this decision, the worker was entitled to a 10% pension up to January 28, 2006, and a 20% pension afterwards.

[91] For these reasons, we are denying the worker's appeal for temporary disability benefits after the incident of May 7, 2005.

(f) Entitlement to lost time benefits during physiotherapy treatment?

[92] The record shows that the Board allowed payment for 12 physiotherapy treatments from October 31, 2007 to February 25, 2008. The worker argues that he is entitled to lost time benefits during that period.

[93] The issue with regard to lost time benefits is not the fact that he was receiving treatment, but whether or not the worker was compensated according to his level of disability and his circumstances pertaining to rehabilitation.

[94] As found earlier, the worker was in receipt of a 20% pension for a permanent disability. This is in accordance with the provisions of section 45 of the pre-1989 Act.

[95] It was also found that, given that the worker was sponsored in a rehabilitation program and that he was able to return to work for five years without a wage loss, he was not entitled to further temporary benefits. The loss of that job is not associated with the worker's injury of October 5, 1988.

[96] Given the above facts, we are unable to consider allowing additional benefits during the period when the worker was undergoing physiotherapy treatment in 2007 and 2008.

[97] However, it must be said that the Panel's finding that the worker was not entitled to additional benefits during the period while receiving physiotherapy, while in receipt of a 20% pension, should not affect the pension examination which we ordered earlier. In other words, we are saying that, considering the worker's argument, based solely on the fact that he was receiving physiotherapy, we are denying further benefits. We are leaving open the issues relating to the level of the worker's disability after January 28, 2006, which we direct the Board to determine.

(g) Entitlement to further LMR?

[98] We are also denying the worker's request for a further Labour Market Re-entry program.

[99] The record shows that the Board had agreed to sponsor the worker on a LMR program (then a VRS program) early in this case. The worker had first dropped out and ongoing full benefits had stopped. However, in a further decision by a Reinstatement Officer in May 1992, it was agreed to sponsor the worker in such a plan. After three years of training, the worker earned a diploma as child and youth worker. The worker obtained employment at a halfway house and was able to remain in that employment for five years. This shows that the worker has successfully completed the program and that he had achieved his pre-injury level of earnings.

[100] The worker states that he lost his job after he was diagnosed with PTSD and depression. He also encountered alcohol and substances problems and was subsequently placed on one-year house arrest for a criminal offence.

[101] Therefore, the circumstances surrounding the worker's separation from his employment at the halfway house do not show an association to his injury of October 5, 1988. The worker has taken action against the employer and apparently reached some settlement.

[102] Moreover, Board policy in OPM Document No. 19-03-04 "Entitlement to LMR Plans" states that, "generally workers are provided with only one LMR Plan." Unless it were shown that the initial LMR Plan was not appropriate, we could not make an exception to this policy provision. In this case, the plan to reintegrate the worker to the workplace as a child and youth worker was successful.

DISPOSITION

[103] The appeal is disposed of as follows:

1. The worker's pension rating of 10% of May 2, 1990 is appropriate.
2. The worker's 20% rating after January 28, 2006 is appropriate. The Board is directed to conduct a new pension assessment of the worker.
3. Entitlement is denied for Chronic Pain Disability.
4. Entitlement is denied for a clothing allowance for the years 1997 to 2004.
5. Entitlement is denied for lost time benefits after May 7, 2005.
6. Entitlement is denied for lost time benefits for the period of physiotherapy treatment in 2007 and 2008.
7. Entitlement is denied for further LMR services.

DATED: February 3, 2012

SIGNED: J.G. Bigras, M.P. Trudeau, K. Hoskin